

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate the Safety and Efficacy of Pumitamig in Combination With Chemotherapy Versus Nivolumab in Combination With Chemotherapy in Participants With Previously Untreated Advanced or Metastatic Gastric, Gastroesophageal Junction, or Esophageal Adenocarcinoma **Short Title/Acronym:** ROSETTA Gastric-204 **Protocol Number:** CA266-0004 **NCT Number:** NCT07221149 **Posting ID:** 369837200664383257 **Sponsor/Company:** Bristol Myers Squibb **Investigational Product:** Pumitamig (BNT327/BMS986545) **Phase of Development:** Phase 2, Phase 3 **Trial Status:** Not Yet Recruiting **Medical Monitor:** Bristol-Myers Squibb Clinical Operations (clinical.trials@bms.com)

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the safety and efficacy of Pumitamig in combination with chemotherapy versus Nivolumab in combination with chemotherapy in participants with previously untreated advanced or metastatic gastric, gastroesophageal junction, or esophageal adenocarcinoma. * **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Duration of Response (DOR), Time to Response (TTR), and overall safety/tolerability of the regimen.

2.2 Background & Rationale Advanced gastric, gastroesophageal junction (GEJ), and esophageal adenocarcinomas have high unmet medical needs. While PD-1 inhibitors like Nivolumab plus chemotherapy have become the standard of care for first-line treatment in certain PD-L1 populations, outcomes remain limited. Pumitamig (BNT327/BMS986545) is a novel bispecific immunomodulator candidate combining PD-L1 checkpoint inhibition with VEGF-A neutralization. Preclinical and early-phase data suggest that simultaneous targeting of the VEGF pathway and immune checkpoints provides synergistic antitumor activity, aiming to improve survival outcomes compared to standard PD-1 inhibition alone.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Active Comparator (Nivolumab + Chemotherapy) * **Blinding:** Double-blind (Blinded) * **Randomization:** Randomized

3.2 Planned Enrollment & Duration * **Target \$\$\$:** Approximately 690 participants * **Number of Sites:** 86 global locations * **Estimated**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants must be previously untreated with systemic treatment for advanced/metastatic disease, histologically or cytologically confirmed advanced or metastatic gastric cancer (GC), gastroesophageal junction adenocarcinoma (GEJC) or distal esophageal adenocarcinoma (EAC). GEJ involvement can be confirmed via biopsy, endoscopy, or imaging. 2. Participants must have a documented programmed cell death-ligand 1 (PD-L1) $\geq 1\%$. 3. Participants must have documented human epidermal growth factor receptor 2 (HER2)-negative cancer, as determined according to local guidelines. 4. Participants must have measurable disease as defined by RECIST v1.1.

4.2 Exclusion Criteria 1. Participants must not have untreated known central nervous system (CNS) metastases. 2. Participants must not have significant cardiovascular disease, such as myocardial infarction, unstable angina, arterial thrombosis, cerebrovascular accident within 6 months prior to randomization, uncontrolled hypertension (≥ 160 mmHg systolic, ≥ 100 mmHg diastolic mm Hg) despite optimal medical management, or congenital long QT syndrome. 3. Participants must not have evidence of major coagulation disorders (eg, hemophilia). 4. Participants must not have a history of deep vein thrombosis, pulmonary embolism, or any other significant thromboembolism within 3 months prior to randomization, unless the participant has been fully treated (eg, inferior vena cava filter placed) and/or adequately anticoagulated on a prophylactic dose. 5. Participants must not have a history of abdominal fistula or gastrointestinal (GI) perforation within 6 months of randomization. 6. Participants must not have had major surgery, open biopsy, or significant traumatic injury within 28 days prior to randomization, or anticipation of the need for major surgery during the course of study intervention. 7. Other protocol-defined Inclusion/Exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen * Dosage/Frequency: Pomitinib administered via infusion in combination with a standard chemotherapy regimen versus Nivolumab (ATC Code: L01FF01; Trade Names: Opdivo, Opdualag) administered with the same standard chemotherapy. * **Route of Administration:** Intravenous (IV) * **Duration of Treatment:** Until disease progression, unacceptable toxicity, or study completion.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care medications (e.g., antiemetics, pain management).
* **Prohibited:** Concurrent systemic anticancer therapies outside the study protocol, concurrent live vaccines, or treatment with other investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Imaging (RECIST 1.1), PD-L1 & HER2 Testing
Baseline	Day 0	Randomization, First Infusion Dose
Interim	Treatment Cycles	Safety Labs, Efficacy/Tumor Assessment (Imaging), Adverse Event monitoring
End of Study	Variable	Final Vital Signs, Exit Interview, Safety follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. **Objective Response (OR)** (confirmed complete response [CR] or partial response [PR]) by Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 per investigator assessment: Phase 2. 2. **Progression Free Survival (PFS)** by RECIST v1.1 per blinded independent central review (BICR): Phase 3. 3. **Overall Survival (OS)**: Phase 3.

7.2 Secondary Outcomes 1. **Progression Free Survival (PFS)** by RECIST v1.1 per investigator assessment. 2. **Duration of Response (DOR)** (CR or PR) by RECIST v1.1 per investigator assessment. 3. **Time to Response (TTR)** (CR or PR) by RECIST v1.1 per investigator assessment.

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Standard oncological collection methods (CTCAE gradings), monitored via regular site visits and patient reports.
 - **Serious Adverse Events (SAEs):** Required reporting within 24 hours to the sponsor and regulatory bodies.
 - **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee assigned to ensure continuous assessment of participant safety and trial integrity.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) population for all efficacy outcomes. Safety Set for evaluating safety events.

- **Sample Size Justification:** \$N=690\$ sufficiently powered to detect clinically meaningful superiority bounds in OS and PFS for the experimental group.
 - **Handling of Missing Data:** Standard oncological time-to-event handling (e.g., Kaplan-Meier estimation with right-censoring).
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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
 - **Monitoring Plan:** Scheduled onsite and remote clinical site monitoring by the sponsor.
 - **Audit Trail:** Utilizing secure Electronic Data Capture (eCRF) systems with full 21 CFR Part 11 compliant audit trails and data clarification forms.
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11. Study Identifiers & Governance

- **Primary ID:** CA266-0004
 - **Registry Number:** NCT07221149
 - **Posting ID:** 369837200664383257
 - **Sponsor/Lead Organization:** Bristol Myers Squibb
 - **Study Title:** ROSETTA Gastric-204
 - **Official Regulatory Title:** ROSETTA Gastric-204: A Blinded, Randomized, Phase 2/3 Study of Pumitamig in Combination With Chemotherapy Versus Nivolumab in Combination With Chemotherapy in Participants With Previously Untreated Advanced or Metastatic Gastric, Gastroesophageal Junction, or Esophageal Adenocarcinoma
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12. Clinical Framework (Oncology Specific)

- **Primary Condition / Disease Area:** Untreated Advanced or Metastatic Gastric, Gastroesophageal Junction, or Esophageal Adenocarcinoma
 - **Preferred UMLS Name:** Upper gastrointestinal tract adenocarcinoma (Neoplastic Process)
 - **Diagnosis Threshold:** HER2-negative, PD-L1 $\geq 1\%$
 - **Medical Methodology:** 1L Systemic Chemo-immunotherapy
 - **Optimization Target:** Objective Response (OR), Overall Survival (OS), and Progression-Free Survival (PFS)
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13. Study Procedures & Methodology

- **Management Pathway:** Standard 1L Oncology Clinical Pathway

- **Education Protocol (HCP):** Site Initiation Visits (SIV), Good Clinical Practice (GCP), and Investigator Meetings
- **Education Protocol (Patient):** Informed consent discussions, infusion schedule planning, and severe AE self-reporting instructions
- **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) of imaging and sponsor adherence audits

14. Observation & Follow-Up Schedule

- **Total Duration:** Follow-up extending until disease progression, death, or trial end in 2032
- **Onsite Visit Cadence:** Aligned directly with 2- to 3-week infusion cycles
- **Remote Monitoring Frequency:** Inter-cycle telehealth connections as permitted
- **Checklist Review Interval:** Routine imaging/tumor assessment every 6 to 12 weeks according to standard RECIST evaluations

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					